

Protocol

OR/M006

December 2021

Pathway for patients with MRI demonstrated spinal cord compression with suspected underlying malignancy

MRI is available at the weekend at all DGHs except QEH, Kings Lynn (Sat 9-12 only).

These patients need to be worked up urgently and histological diagnosis established as soon as possible.

The starting point to this pathway is the demonstration on MRI of spinal cord compression of non-degenerative origin.

Pathway

Baseline primary investigations to be completed prior to specialty referral:

CT Chest/Abdo/Pelvis – this is an emergent investigation in this condition, as it would be inappropriate to devise a management strategy without this information
PSA or Breast Exam, Myeloma screen
FBC/U&E/LFT/Bone profile/CRP/INR/APTT

1 Patients presenting to DGH:

If confirmed or suspected Primary Malignancy on CT CAP

Case to be discussed with local Oncology/AOS team first to get an idea of prognosis;

WSH/Bedford/ Kings Lynn will have already had AOS input and local MSCCC (Malignant Spinal Cord Compression Coordinator) to follow referral guideline below

Lister AOS+RT is at Mt Vernon; They should have discussed with local Oncology team (Mt Vernon) and local MSCCC to follow referral guideline below

Hinch/Peterborough have AOS and do their RT Mon-Fri in Peterborough; local MSCCC to follow referral guideline below

If no confirmed or suspected Primary Malignancy on CT CAP

- a) Referral to CUH Neurology: Patients with no obvious primary identified on CT CAP, requiring biopsy for Dx

2 Patients presenting to CUH:

If confirmed or suspected Primary Malignancy on CT CAP

- a) Referral to CUH Medics: Patients with likely primary malignant lesion demonstrated on CT chest/abdo/pelvis to be managed as per existing newly diagnosed cancer pathway – arrange Oncology review for prognostication

and determination of optimal management strategy (24/7 service- as prognostication required for surgical decision making);

If no confirmed or suspected Primary Malignancy on CT CAP

- a) **Referral to CUH Neurology:** Patients with no obvious primary identified on CT CAP, presenting to CUH ED, requiring further workup
 - i.) Neurology team to please alert Cancer of Unknown Primary team via EPIC referral (CUP CNS is Helen Whatling Bleep 159-409) for assistance with prognostication
 - ii.) Percutaneous biopsy should be considered to aid in prognostication in ambulant patients with stable neurology, patients with severe neurological deficit who have not been ambulant for 48 hours or longer as a result of the deficit, and in any other patient in whom histological confirmation of diagnosis is desirable
- b) Patients with prognosis under 6 months would not be candidates for surgical intervention
 - i.) Consult neurosurgery on call team for consideration of surgery in: Patients with estimated prognosis in excess of 6 months, who would consider surgical intervention, with:
 - 1. worsening Neurology and good PS,
 - 2. non-ambulant, but with acute (<24 hour) change in neurology, and good PS prior to acute deterioration,
 - 3. mechanical back pain limiting ambulation, with SINS score >6

3 CUH Neurosurgery

Patients accepted by neurosurgery on call team following consultation by relevant MSCCC (Malignant Spinal Cord Compression Coordinator) – these would generally be patients with:

Patients with estimated prognosis in excess of 6 months, who would consider surgical intervention, with:

- i.) worsening Neurology and good PS,
- ii.) non-ambulant, but with acute (<24 hour) change in neurology, and good PS prior to acute deterioration,
- iii.) mechanical back pain limiting ambulation, with SINS score >6

RT not recommended without histological diagnosis (unless clear primary identified on CT or there is disseminated malignancy)

For Ongoing management, please refer to the 'Spinal Cord Compression Pathway for patients known to have Malignancy' protocol

Patients whose pain or neurological symptoms get worse when they stand or walk, or with a SINS score > 13, should be on bed rest until their images and clinical presentation have been reviewed by a spinal surgeon

4 Monitoring compliance with the effectiveness of this document

- a) Process for monitoring compliance and effectiveness: Review of Safety Learning Events, as recorded on the Trust QSI reporting system, for non-compliance and the results presented to the Patient Safety & Governance group - the minutes of these meeting are retained for a minimum of 5 years.
- b) Standards/Key Performance Indicators: This process forms part of a quality system accredited to International Standard BS EN ISO 9001:2015. The effectiveness of the process will be monitored in accordance with the methods given in the quality manual ([QM1.00](#)).

Equality and diversity statement

This document complies with the Cambridge University Hospitals NHS Foundation Trust service equality and diversity statement.

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Document management

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